

ENGLISH FOR WORK / SEPTEMBER 2026

Home Care & Caregiving English



Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Home care aides, personal care assistants, and caregivers.

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

Open this course on English Ladder

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

acknowledgment

Confirmation that a message or concern has been received, not necessarily resolved.

activities of daily living (ADLs)

Basic personal activities such as dressing, eating, and bathing; the care plan specifies any assistance required.

ambulation

Walking or moving about on foot; a report about ambulation describes mobility rather than diagnosing its cause.

approved channel

An employer-authorized method for communicating particular workplace information.

assigned visit

A scheduled period of support with duties assigned to a particular worker.

authorized contact

A person verified as permitted to receive specified information or take part in specified communication.

authorized task

A duty the worker is permitted to perform under the applicable assignment and procedures.

baseline

A person's usual pattern used as a reference when describing a change.

care plan

The current agreed record of a client's needs, support, and authorized arrangements.

client preference

A person's stated choice about how they want support provided.

clinical advice

Guidance about a health condition or treatment from an appropriately qualified professional.

confidentiality

The obligation to protect private information from inappropriate disclosure.

consent

Agreement to an action, which must be checked rather than assumed.

contingency plan

An approved alternative arrangement used when the original plan cannot proceed as expected.

dietary restriction

A documented limit affecting food choices, which must not be guessed or reinterpreted.

escalation

Passing a concern to the appropriate authorized person or urgent response process.

follow-up owner

The person explicitly responsible for taking an unresolved matter to its next step.

grooming

Personal care relating to appearance and cleanliness, such as caring for hair; assistance depends on the person's wishes and plan.

handoff

The transfer of relevant information and responsibility between workers or teams.

identity verification

The approved process for establishing who a person contacting the service actually is.

intake record

An approved record of food or drink consumed, with estimates identified as estimates.

light housekeeping

Agreed routine household tasks such as washing dishes and laundry, distinct from major cleaning or repairs.

meal preference

A person's choice about what or how they would like to eat.

medication reminder

An authorized prompt about medicine, distinct from administering it or choosing a dose.

observation

Something a worker directly sees, hears, or otherwise notices.

pending action

A task or decision that remains unresolved and needs a next step.

personal space

The physical and conversational distance a person wants around them.

read-back

Repeating key information to confirm that it has been heard and understood correctly.

reassurance

Language that offers support without promising an outcome you cannot guarantee.

referral

Directing a question or concern to a person qualified or authorized to handle it.

refusal

A person's decision not to accept a proposed action or service.

reported symptom

A person's own account of how they feel, distinguished from a worker's diagnosis.

scope of duties

The limits of the tasks a worker is authorized to carry out.

service disruption

An interruption or change that affects the planned delivery of support.

substitution

An item proposed or used in place of another item.

visit window

The agreed period during which a scheduled visit is expected to begin.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Check expectations before starting

My assignment lists ____; were you expecting something else?

Could we check whether ____ was arranged separately?

I can confirm ____, but I still need to check ____.

A care plan records agreed support; a client's immediate request may differ. Acknowledge the need, identify what your current assignment actually says, and ask the person who can clarify it. An omission is not proof that nobody arranged support.

Word combinations in this lesson

review the care plan: check the current agreed support before starting

confirm the visit window: establish the expected arrival period

respect a preference: accommodate a choice within the agreed support

clarify the assignment: ask exactly which duties are authorized

Offer choices without applying pressure

Would you prefer ____ or ____?

You can say no; I need to explain ____ so you can choose.

I'll give you ____, then check whether you would like ____.

A polite question is not meaningful choice if a refusal is immediately ignored. State available options neutrally, allow an answer, and separate the person's preference from what the schedule allows.

Word combinations in this lesson

ask permission: check that the person agrees before acting

offer a choice: present manageable options without pressure

respect personal space: avoid moving closer without agreement

accept a refusal: acknowledge a no and clarify what happens next

Separate what you observed from what you inferred

At ___, I noticed ___; the client said ___.

Compared with my usual visits, ___.

I cannot determine the cause; I am reporting ___.

An observation is something you directly noticed; a reported symptom is the person's account. A baseline is their usual pattern, not a diagnosis. Do not convert one unfamiliar behavior into a clinical conclusion or delay urgent action to finish a language exercise.

Word combinations in this lesson

notice a change: identify something different from the usual pattern

report an observation: describe what you directly saw or heard

quote the client's words: distinguish their report from your conclusion

escalate a concern: pass it to the authorized person or emergency process

Decline the decision, not the person

I can help with ___, but I am not authorized to ___.

That question needs ___; may we contact them?

I don't want to guess about ___. Let's check the approved route.

A boundary is clearest when it names the specific decision you cannot make and the next person who can help. Do not suggest a dose, interpret a prescription, or let a relative's confidence substitute for authorized guidance.

Word combinations in this lesson

stay within your role: perform only duties you are authorized to carry out

refer a clinical question: connect the person with an appropriate professional

check the current instructions: verify approved guidance rather than rely on memory

decline a request respectfully: state a limit and offer a suitable next step

Clarify food choices without giving nutrition advice

The plan lists ____; the requested substitute still needs ____.

Is that a preference, or is it part of the documented restriction?

I can offer ____ now, or help check ____.

A preference describes what someone wants; a restriction limits what is appropriate under their plan. The caregiver should not invent a clinical reason for a restriction or promise that a substitute is suitable before it has been checked.

Word combinations in this lesson

check a restriction: verify a documented limit before discussing alternatives

offer an approved alternative: suggest an option consistent with current instructions

record what was consumed: describe the actual amount without guessing

distinguish preference from restriction: separate a personal choice from a required limit

Protect privacy without sounding dismissive

Before discussing ____, I need to verify ____.

I can pass on your request without sharing ____.

Please use ____ so the authorized person can check ____.

Knowing a client's name or address is not proof of authorization. Family relationships do not tell you exactly what may be shared. Use the organization's verification procedure, avoid confirming private facts unnecessarily, and do not promise access you cannot authorize.

Word combinations in this lesson

verify the caller's identity: establish who is asking before discussing private details

check permission to share: confirm what the person is entitled to receive

use the approved channel: communicate through the employer's authorized method

limit the disclosure: share only information appropriate to the authorized purpose

Transfer a pending action without losing ownership

Completed: ___. Still pending: ___.

___ is handling the follow-up; please check ___ by ___.

Could you read back the status and the person responsible?

A handoff is not just a list of events. The receiving person needs to know what matters for their visit, which facts remain uncertain, and who is acting. A read-back catches misunderstandings before 'requested' becomes 'confirmed'.

Word combinations in this lesson

give a concise handoff: transfer the relevant facts and unfinished work

name the follow-up owner: identify who will take the next action

read back the key detail: repeat it to check shared understanding

close the communication loop: obtain confirmation that the message was received and understood

Repair trust with a bounded promise

I'm sorry this affects ___. What we know now is ___.

The current estimate is ___; it is not yet guaranteed.

___ will update you by ___ about ___.

People need practical information during delays. Separate the estimated arrival from the promised next contact, and explain the escalation route if the update is missed. Apologizing should not lead to an invented guarantee or an unauthorized change to support.

Word combinations in this lesson

acknowledge the impact: recognize how a delay affects the person

give a realistic update: state what is known without inventing an arrival promise

follow the missed-visit procedure: use the organization's established response

confirm the next contact: say who will update the person and when

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Confirming the visit and the care plan

You arrive at 9:00 for a two-hour visit. Your assignment lists breakfast and light household tasks. The client expects transport to a 10:00 appointment, but transport is not listed and you have no authorization. The coordinator is available by phone.

Your appointment is at ten, and I understand why you expected a ride. My assignment today lists breakfast and light household tasks, but it does not include transport. May I contact the coordinator now to check what was arranged? I cannot confirm a ride until we have that information.

Try it again: The coordinator says a separate transport service may have been contacted but needs five minutes to confirm. Explain the uncertainty and agree what to do while waiting.

AI extension: build and retrieve vocabulary from this case.

02 | Consent, choice, and respectful assistance

The client declines help arranging clothes before a visitor arrives. The visit has thirty minutes left. Choosing clothes is part of the agreed support, but the client prefers ten quiet minutes first. There is no immediate safety concern.

You would like some quiet time before choosing clothes. We have thirty minutes left in today's visit. Would you prefer ten minutes alone and then help with the clothes, or would you rather leave that task today? Either way, I will respect your choice and confirm what support you want next.

Try it again: The client chooses to leave the task today. Accept the decision and agree which remaining support, if any, they want.

AI extension: build and retrieve vocabulary from this case.

03 | Reporting a change without diagnosing

During a fictional routine visit, the client says, 'I feel more tired than yesterday.' You notice they stop twice while describing breakfast, which is unusual for your visits. You are not qualified to diagnose. No emergency signs are specified; use the local procedure for reporting changes and urgent concerns.

The client told me, 'I feel more tired than yesterday.' During our conversation, they paused twice before finishing their account of breakfast. That differs from my usual visits, but I cannot identify the cause. I am reporting the change through our agreed process and need the authorized clinician's direction on the next step.

Try it again: The supervisor asks whether the client is 'just tired.' Restate the facts and your knowledge limit without agreeing to an unsupported explanation.

AI extension: build and retrieve vocabulary from this case.

04 | Boundaries around medicines and clinical questions

Your fictional assignment permits reminders only, not administering medicines or recommending doses. The client asks whether to double a missed dose. You do not know the medicine details. The care plan identifies a nurse contact for medication questions.

I understand you want to know what to do about the missed dose. My role permits reminders, but I cannot recommend a dose or tell you to double it. Let's contact the nurse using the number in your care plan. I can help communicate your question without making the medication decision myself.

Try it again: The client says a neighbor gave an answer already. Explain why you still cannot endorse that advice and offer to help contact the appropriate professional.

AI extension: build and retrieve vocabulary from this case.

05 | Meals, preferences, and unresolved restrictions

The plan lists two approved breakfast options. The client dislikes the first and asks for a third food that is not on the list. You do not know whether it meets the documented restrictions. The second approved option is available. The coordinator can clarify the third option.

You don't want the first breakfast option today. The second option in your plan is available, or I can ask the coordinator about the food you requested. I don't know whether that substitution fits your documented restrictions, so I should not describe it as approved. Which of those next steps would you prefer?

Try it again: The client asks why the restriction exists. You do not know the clinical reason; acknowledge the question and refer it without making one up.

AI extension: build and retrieve vocabulary from this case.

06 | Privacy and family communication

An unfamiliar caller says she is the client's niece and asks what happened during the visit. The contact record is unavailable to you at that moment. You cannot verify her identity or permission. Your coordinator can handle the request through the approved process.

I understand you are concerned about your aunt. Before discussing visit details, I need to verify the contact arrangement through our approved process. I cannot confirm private information on this call. Please contact the coordinator through the agency's usual number, or I can pass on your request without sharing the client's details.

Try it again: The caller says the check is insulting. Acknowledge the frustration and explain that the same process protects every client's privacy.

AI extension: build and retrieve vocabulary from this case.

07 | A handoff the next person can use

At 11:00 you finish a fictional visit. Breakfast support and laundry are complete. A transport query for tomorrow's 10:00 appointment is still pending with the coordinator. The incoming caregiver starts at 15:00. The coordinator, not the incoming caregiver, is arranging transport.

Breakfast support and laundry are complete. Transport for tomorrow's ten o'clock appointment is still unconfirmed; the coordinator owns that follow-up. Your visit starts at three today. Please check the coordinator's update through the approved channel, and do not describe the ride as booked until confirmation arrives. Could you read back the pending item and who is handling it?

Try it again: The incoming caregiver assumes that checking the update means personally booking another ride. Correct the misunderstanding before a duplicate request is made.

AI extension: build and retrieve vocabulary from this case.

08 | Delays, complaints, and reliable follow-up

Your fictional 9:00 visit will be delayed. At 8:50, dispatch estimates arrival between 9:30 and 9:45 but cannot guarantee it. The client had planned breakfast support at 9:00. Dispatch is checking an approved backup arrangement and will update the client by 9:10.

I'm sorry this delay affects your planned breakfast support. Dispatch currently estimates arrival between nine thirty and nine forty-five, but that is not a guaranteed time. They are checking the approved backup arrangement and will update you by nine ten. I will not describe the backup as confirmed until dispatch verifies it.

Try it again: At 9:10 no update has arrived. Use the approved follow-up route and describe the missed update accurately without assuming a backup was cancelled.

AI extension: build and retrieve vocabulary from this case.

Use the language responsibly

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

BLS: Home Health and Personal Care Aides

<https://www.bls.gov/ooh/healthcare/home-health-aides-and-personal-care-aides.htm>

CDC: Plain Language Materials

<https://www.cdc.gov/health-literacy/php/develop-materials/plain-language.html>

BLS: Personal Care Aides Occupational Requirements

<https://www.bls.gov/ors/factsheet/pdf/personal-care-aides.pdf>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Build vocabulary and collocations

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Home Care & Caregiving English

Professional audience: Home care aides, personal care assistants, and caregivers

Scope: Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

Lesson 01: Confirming the visit and the care plan

Original fictional case: You arrive at 9:00 for a two-hour visit. Your assignment lists breakfast and light household tasks. The client expects transport to a 10:00 appointment, but transport is not listed and you have no authorization. The coordinator is available by phone.

Language workshop: Check expectations before starting

Communication goal: Name the mismatch between the assignment and the client's expectation without blaming either person.

Language guidance: A care plan records agreed support; a client's immediate request may differ.

Acknowledge the need, identify what your current assignment actually says, and ask the person who can clarify it. An omission is not proof that nobody arranged support.

Useful frames (complete the gaps with case facts): My assignment lists ____; were you expecting something else? / Could we check whether ____ was arranged separately? / I can confirm ____, but I still need to check ____.

Vocabulary: care plan: The current agreed record of a client's needs, support, and authorized arrangements. / assigned visit: A scheduled period of support with duties assigned to a particular worker. / scope of duties: The limits of the tasks a worker is authorized to carry out. / client preference: A person's stated choice about how they want support provided.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. The appointment is at 10:00, and you remember discussing transport last week, but you do not know whether it was booked.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: The coordinator says a separate transport service may have been contacted but needs five minutes to confirm. Explain the uncertainty and agree what to do while waiting.

Model for comparison AFTER my attempt, not a response to give me first: Your appointment is at ten, and I understand why you expected a ride. My assignment today lists breakfast and light household tasks, but it does not include transport. May I contact the coordinator now to check what was arranged? I cannot confirm a ride until we have that information.

END REFERENCE

TASK: Retrieve occupational vocabulary

1. Select four terms from the reference. Explain each in plain English, give two natural collocations and quote or create a short workplace example. Label invented examples. Contrast each term with a plausible confusable term; acknowledge regional variation.
2. Present one role-labelled exchange with one missing word or phrase and four choices A-D. Exactly one choice must fit the stated facts and grammar. Stop and wait for my answer before revealing it.
3. Explain all four options, then invite me to read the completed exchange aloud. Offer a sentence frame for optional spoken practice, never an open-ended writing assignment.
4. Repeat for the other three terms, one question at a time. Vary the correct letter and the situation; preserve the reference facts. If a choice is ambiguous, replace the item.
5. Finish with two new four-choice collocation questions for later review, with a clearly separate answer key. Do not claim to schedule a reminder.

END PROMPT